As of: March 21, 2025
Received: March 14, 2025
Status: [redacted]
Tracking No. m89-am1w-g8js
Comments Due: March 15, 2025
Submission Type: Web

PUBLIC SUBMISSION

Docket: NSF_FRDOC_0001
Recently Posted NSF Rules and Notices.

Comment On: NSF_FRDOC_0001-3479
Request for Information: Development of an Artificial Intelligence Action Plan

Document: NSF_FRDOC_0001-DRAFT-1480
Comment on FR Doc # 2025-02305

Submitter Information

Email: [redacted]
Organization: American College of Emergency Physicians

General Comment

On behalf of our nearly 40,000 members, the American College of Emergency Physicians (ACEP) appreciates the opportunity to provide input on the request for information on the development of an artificial intelligence (AI) action plan. Please see the attached document for our full comments.

Attachments

ACEP AI Action Plan RFI Response_

March 14, 2025

Kirk Dohne
Acting Director
Networking and Information Technology Research and Development (NITRD)
National Coordination Office (NCO)
490 L'Enfant Plaza SW, Suite 8001
Washington, DC 20024, USA

Michael Kratsios
Acting Director
White House Office of Science and Technology Policy
1600 Pennsylvania Ave
Washington, DC 20500

RE: Request for Information: Development of an Artificial Intelligence Action Plan

Dear Acting Directors Dohne and Kratsios:

On behalf of our nearly 40,000 members, the American College of Emergency Physicians (ACEP) appreciates the opportunity to provide input on the request for information on the development of an artificial intelligence (AI) action plan. AI has the opportunity to revolutionize the practice of emergency medicine with the potential to provide highly personalized care, while reducing physician burdens and improving the physician-patient relationship.

As a leader in advancing the responsible use of AI in emergency medicine, ACEP is engaged in policy development work on AI through our AI Task Force, Health Innovation Technology Committee, and Disaster Medicine Committee. We are committed to identifying and exploring opportunities and threats posed by new developments in artificial intelligence to the emergency medicine workforce as well as how new technologies such as machine learning and AI can be leveraged to innovate approaches to quality and safety work. As this work gets further underway, we welcome an opportunity to participate in a more nuanced development of guidelines.

ACEP believes that health care AI must be designed, developed, and deployed in a manner which is ethical, responsible, accurate, transparent, and evidence-based. The use of AI in health care delivery requires clear national governance policies to regulate and continuously update its adoption and utilization. To that end, ACEP recommends three major policy priorities to ensure that the U.S. remains the global leader in AI-driven health care innovation while enhancing emergency patient care and physician efficiency.

<u>Ensuring Clinician Oversight in AI Integration</u>

While ACEP believes that AI tools can be helpful in enhancing patient care, use of AI in health care delivery requires clear, transparent, and specific national governance policies to regulate its adoption and utilization, with strict compliance standards. AI should support, not replace, clinical judgment, with emergency physicians maintaining ultimate oversight to ensure safe and effective patient care. AI-driven clinical decision support tools must be designed with built-in safeguards that allow emergency physicians to verify recommendations and intervene when necessary.

Emergency physicians are best equipped to identify the most appropriate uses of AI-enabled technologies in emergency medicine practice, and they should be the ultimate drivers of setting the standard of appropriate AI use in delivery of emergency care. To that end, emergency physicians, clinical experts, and informaticists should be the leading voice in developing best practices for AI integration in emergency medicine, including, but not limited to automated signal processing, speech recognition, AI scribes, and predictive analytics. Any clinician using AI support tools should use their clinical expertise to oversee and validate AI-assisted diagnoses, treatment recommendations, and produced notes.

Developing Best Practices and Risk Mitigation Strategies

As stated above, emergency physicians should be the ultimate drivers of setting the standard of appropriate AI use in delivery of emergency care. In addition to describing best practices, emergency physicians should establish guidelines for AI-driven documentation, including large language model (LLM)-based patient discharge summaries, autonomous coding, and against medical advice (AMA) risk documentation, ensuring emergency physician review before finalization. AI risk management should minimize potential negative impacts of health care AI systems while providing opportunities to maximize positive impacts, such as reducing administrative burden and time away from patient care. Whilst AI tools can be helpful in minimizing burnout, health care practices and institutions should not utilize AI systems or technologies that introduce overall or disparate risk that is beyond their capabilities to mitigate. Implementation and utilization of AI should avoid exacerbating clinician burden and should be designed and deployed in harmony with the clinical workflow.

Further, as implementation of AI-enabled tools and systems increases, it is essential that use of AI in health care be transparent to both patients and physicians. Transparency is one of the most critical elements to build trust in new AI-enabled technologies and serves to help both physicians and patients understand when and what they are engaging with when AI is involved in the care experience. We recommend requiring transparency for AI-enabled technologies with clinical applications that contribute to medical decision making and suggest that the Administration develop transparency frameworks for other uses of AI, such as AI with administrative applications, with careful consideration of liability concerns and data privacy protections.

Advancing Data Standardization and Interoperability

A longstanding priority of the Department of Health and Human Services (HHS), establishing a foundation of standards to build interoperable health information technology (HIT) across health systems allows delivery of the highest quality of patient care to remain our focus by increasing ease of communication through data standardization. This concept should extend to the development and integration of AI tools in health care delivery. Standards and benchmarks around AI development should be established and evaluated for continuous improvement to ensure reliability, interoperability, and ease of use. Data integrity, patient privacy, and production of clear, interpretable AI outputs that physicians can trust must be incorporated and given utmost consideration when interoperability standards are created and assessed.

Conclusion and Further Considerations

AI holds tremendous potential to transform emergency medicine, with a significant opportunity to enhance patient care, improve operational efficiency, and strengthen the physician-patient relationship. However, rigorous standards for responsible AI implementation, compliance, and risk mitigation, with emergency physicians leading the development of best practices and ensuring physician oversight, should be of the utmost priority to ensure patient safety, quality of care, and ethical responsibility.

We appreciate the Administration's request for information on the development of an AI action plan, and we look forward to working with you to prioritize focus on health care AI and to promote policies that ensure the safety and wellbeing of our patients. Should you have any questions or wish to discuss further, please contact Erin Grossmann, ACEP's Regulatory and External Affairs Manager, at [redacted]

Sincerely,

Alison Haddock, MD, FACEP
ACEP President